BASELINE → TREATMENT → FOLLOW-UP

Checks before, during and after treatment

Some tests are required for everyone; electrocardiogram (ECG)/QT, pancreatic enzymes, international normalised ratio (blood-clotting test) (INR) and extra checks depend on risk, symptoms or other medicines. There is no single mandatory imaging or tumour-marker interval for all patients.

Check	Before starting	First six weeks	Up to two months	Later cycles	After / stopping
Liver blood tests	Baseline liver profile	Every two weeks	Every two weeks	Monthly or as clinically indicated	Follow any abnormal results until reviewed / resolved
Blood pressure	Baseline; control existing hypertension	Weekly	Before each cycle after week six; extra if needed	Before each cycle or as indicated	Follow the team's plan; do not stop blood pressure (BP) tablets yourself
Blood count and kidneys	Baseline full blood count (FBC) and renal profile	Regular tests as arranged	Continue scheduled monitoring	Regular tests as arranged	Review residual blood-count, fluid or kidney problems
Thyroid and additional tests	Thyroid function; ECG/QT if at risk; pancreatic enzymes if indicated	Thyroid / pancreas tests if indicated; ECG / heart-rate review if needed	If on warfarin: close INR checks	Further tests according to symptoms and medicines	Follow abnormalities and symptoms individually
Symptoms and cancer response	Agree aim, baseline symptoms and review plan	Review hands/feet, mouth, bowel, appetite, energy and wounds	Review tolerability and any interruptions	Scans / markers according to the individual cancer plan	Agree follow-up, next treatment or supportive care

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00244 v5 · Regorafenib Monotherapy · revised 25 March 2025 · EMA Stivarga · current product information and patient leaflet · checked 5 November 2026

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Regorafenib (Stivarga) monotherapy: treatment and checks

Previously treated metastatic colorectal cancer

Take the prescribed dose once daily on Days 1–21, then take no regorafenib on Days 22–28. Each cycle lasts 28 days. Take tablets at the same time after a light, low-fat meal. Start another cycle only when the team authorises it.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First six weeks	Up to two months	Later cycles	After / stopping
Liver blood tests NCCP 00244	Baseline liver profile	Every two weeks	Every two weeks	Monthly or as clinically indicated	Follow any abnormal results until reviewed / resolved
Blood pressure NCCP 00244	Baseline; control existing hypertension	Weekly	Before each cycle after week six; extra if needed	Before each cycle or as indicated	Follow the team's plan; do not stop BP tablets yourself
Blood count and kidneys NCCP 00244	Baseline FBC and renal profile	Regular tests as arranged	Continue scheduled monitoring	Regular tests as arranged	Review residual blood-count, fluid or kidney problems
Thyroid and additional tests NCCP 00244	Thyroid function; ECG/QT if at risk; pancreatic enzymes if indicated	Thyroid / pancreas tests if indicated; ECG / heart-rate review if needed	If on warfarin: close INR checks	Further tests according to symptoms and medicines	Follow abnormalities and symptoms individually
Symptoms and cancer response NCCP 00244	Agree aim, baseline symptoms and review plan	Review hands/feet, mouth, bowel, appetite, energy and wounds	Review tolerability and any interruptions	Scans / markers according to the individual cancer plan	Agree follow-up, next treatment or supportive care

What each check is for

1

21 tablet days, then seven rest days

28-day cycle

Regorafenib is a targeted tablet treatment for colorectal cancer that has spread, after previous treatments or when those treatments are unsuitable. The aim is to control the cancer. Your team discusses the likely benefit, side effects, alternatives and your priorities.

Ask / **act:** Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00244

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.34